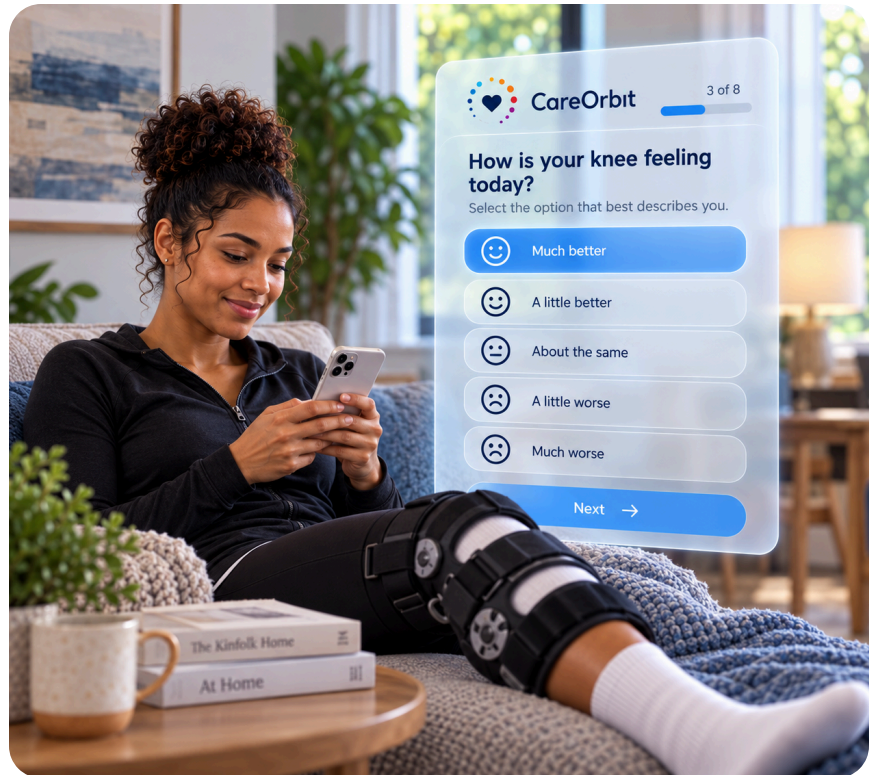


ASSESS

Patient-reported data, *without the chasing.*

Assess adds structured patient input to any orbit, from light check-ins to formalized eScreening and PROMs reporting, and it also runs entirely on its own, with no orbit required. Scores return structured, routed by role and urgency instead of waiting in a portal inbox. No app, no login, no new FTEs.

- Pre-built assessments for major requirements, and a flexible builder for your own.
- Paired with complimentary CareOrbit Engage support around every assessment.



Assessments arrive by text or email and open in the patient's own browser on any device. No downloads or logins ever required.

What Assess runs

- **Inside any orbit.** Symptom and progress check-ins scheduled to the clinical pathway, answered from the guide patients already open.
- **PROMs reporting programs.** Built for the pre-op and post-op survey windows that reporting programs actually enforce.
- **Standalone eScreening.** Validated instruments on schedule, no orbit required. Deployed across VA medical centers nationwide.
- **Routed, not parked.** Rising risk surfaced early, with scores routed by role and urgency instead of a portal inbox.

ESCREENING, PROVEN IN THE VA

Published two-cohort study, electronic versus paper screening

19+

VA medical centers adopted eScreening by 2022, 18 more in progress

VA Diffusion of Excellence; Pittman et al., 2017.

1,372

veterans in the published two-cohort study

Higher

completion and access than paper screening

Faster

connection to care and suicide risk documentation

Required reporting, *and screening that pays its way.*

PROMs and screening are no longer optional. CMS has put dates and payment on the calendar, and the same instruments that satisfy the mandate can support CPT-coded services, grant-funded programs, and value-based contracts when capture rates hold.

The PROMs mandate, in dates

CMS IQR, hip and knee first

01	Jul 2024: collection began	Pre-operative PROMs capture required for elective hip and knee replacement. (CMS IQR / AAOS)
02	50%: required capture	Minimum share of eligible patients with complete pre- and post-op surveys and risk variables. (CMS IQR / AAOS)
03	FY 2028: payment at risk	Hospitals that miss reporting requirements face an annual payment reduction. (J. Arthroplasty, 2024)
04	2027: expansion arrives	Mandatory reporting extends to outpatient and ambulatory settings after a voluntary phase. (J. Arthroplasty, 2025)
05	Completion that survives the gap	Reminders ride the channel patients already respond to, protecting capture rates through the long post-op windows.
06	CPT-coded services supported	Structured results into the screening services your clinicians already perform and document.

REIMBURSEMENT AND FUNDING

Screening that pays its way

Billable screening support

Delivery and documentation behind CPT-coded services, such as 96127, 96160/96161, and G0444.

Capture rates that hold

Reminders patients answer, so scheduled screenings are completed and billable, not missed.

Grant initiatives

Scheduled instruments and structured exports support grant-funded programs and community studies.

Value-based reporting

PROMs and screening results, structured for CMS, MIPS, and payer programs.

CPT codes shown as examples. Billing eligibility depends on payer policy and documentation.

Every patient scored, *every score routed*.



An example Assessment Index, as your team sees it: every patient scored, routed by role and urgency.

WHAT YOU GET

Pre-built, flexible, and never a dead end

- **Pre-built assessments.** Ready-made packages for major requirements: CMS PROMs windows, behavioral screening, and annual screenings.
- **Build and deploy your own.** A flexible builder for research and study support, patient information, satisfaction, and even marketing.
- **Paired with Engage.** Complimentary CareOrbit Engage journeys dynamically support patients around each assessment, with preparation, reminders, and education that lift completion and results.
- **Results that go somewhere.** Scores routed by role and urgency, and structured results into your EHR, registry file, or team workflow.

THE DIFFERENCE

Most systems stop when the assessment is submitted. CareOrbit keeps working the result: scores are routed, patients are supported, and the next assessment is already scheduled.

WHERE TEAMS START

Four proven starting points

The screening or PROMs requirement you are on the hook for, and who reports it today.

- **Ortho PROMs.** The CMS hip and knee windows, pre-op and long post-op.
- **In-orbit check-ins.** Light assessments woven into any journey.
- **Behavioral screening.** PHQ-9, GAD-7, and the instruments your program runs.
- **Standalone eScreening.** No orbit required: an order, a list, a QR code, or a text.

INSTRUMENT SUPPORT

Validated instruments, on schedule

Standard instruments configured to your protocol, with scoring and thresholds your team sets.

- PHQ-9 and PHQ-2 depression
- HOOS JR and KOOS JR hip and knee PROMs
- C-SSRS suicide risk screening
- PC-PTSD-5 trauma screening
- Custom check-ins your protocol's questions
- GAD-7 anxiety
- PROMIS Global-10 global health
- AUDIT-C alcohol use
- GAIN substance use and behavioral health
- Custom instruments built to your study or program

Pick one requirement.

Prove it in a quarter.

ZERO DISRUPTION

Nothing added to anyone's day

Issued from the workflow you already run: an order, a patient list, a QR code, or a text.

- **No new FTEs.** Nothing added to anyone's day.
- **Live in a quarter.** From kickoff to first patient.
- **No risk, no lock-in.** And no disruption to your offices.
- **White-label.** Your brand, your voice.
- **HIPAA, SOC 2, HITRUST.** Documentation ready for review.
- **EHR-friendly.** A scheduled flat file, or nothing at all.

WHAT HAPPENS NEXT

Three steps from kickoff to live

- 01 Pick the requirement** WEEK ONE
The screening or PROMs mandate you are on the hook for. We agree on the two or three numbers this has to move.
- 02 We build it** WEEKS TWO THROUGH SIX
Your instruments, your schedule, your brand and thresholds. We build it for your team's review.
- 03 We measure against your baseline** FROM THE FIRST PATIENT
Capture rates, completion, and the reporting numbers you named in week one, evaluated against your baseline.

COMMON QUESTIONS

What your team might ask

- **Does my staff have to chase surveys?** No. Scheduling, reminders, and escalation run automatically. Your team sees scores and acts on the ones that need attention.
- **What does IT have to do?** As little as a scheduled flat file, or nothing at all. HIPAA, SOC 2, and HITRUST documentation is ready for review.
- **Can we bill for this?** Assess supports the delivery and documentation behind CPT-coded screening services. Eligibility depends on payer policy.
- **How fast will we know if it works?** Six weeks to launch, and the first cohort reports capture rates against your baseline inside a quarter.

The next move: 20 minutes discussing *your* assessment needs and *our* platform.

No demo script, no obligation. We hear which assessment needs and requirements are costing your program the most, and give you a brief look at the platform.

BOOK A 20-MINUTE INTRO CALL

Michael Margraf

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